CenterWell Senior Primary Care

Tandem Care

Collaborative Care program performance, The Villages

Period **Jul 1 to Sep 30, 2026** Data as of **Mon, Oct 19** Interview mock-up, illustrative data

Program reach (enrolled of eligible) **46%** Stretch target 50% Watch 12 months to Sep 30, 2026	Referral to initial assessment **16 days** Target 14 days or less Off target Referrals Jul to Sep 2026	Caseload per care manager **70 avg** Band 60 to 80, 1 above Watch Active as of Oct 19, 2026
Response at 6 months **48%** Target 50% or more Watch Enrolled Jan to Mar 2026	Same-day suicide risk check **97%** Target 100% (31 of 32) Act now Jul to Sep 2026	Cost PMPM vs. matched group **–7.6%** Target 5% lower or better On target 12 months to Sep 2026. Range –1% to –14%

Weekly Priorities Ranking

1. Act now **Review one missed same-day suicide risk assessment**

   Patient TC-51627 answered PHQ-9 item 9 positive on Oct 14 with no Columbia (C-SSRS) screen documented that day. Every positive item 9 needs a same-day risk assessment and safety plan.

   Owner **Clinical lead and D. Price** When **Today**

2. Off target **Close 12 psychiatric recommendations open more than 14 days**

   5 of the 12 sit with Dr. Alvarez. Each one is a medication or treatment change the psychiatric consultant made that the PCP hasn't acted on, so the patient stays on a plan that isn't working.

   Owner **Medical director** When **Thursday huddle**

3. Off target **Route new referrals away from Karen Moreno for 30 days**

   84 active patients (band is 60 to 80), a 21-day wait to assessment, and 9 patients with no contact in 30+ days. Thomas Brooks is at 58 and can take the next 10 referrals.

   Owner **Program delivery lead** When **This week**

4. Watch **Raise referral rates on three PCP panels**

   Dr. Whitfield, Dr. Okafor, and Dr. Hale account for 31 of the 46 positive screens that never got a referral. Share the peer comparison at the provider meeting and set up warm handoffs at the visit.

   Owner **Medical director** When **Next provider meeting**

5. Watch **Protect psychiatric caseload review time**

   21 patients are 10+ weeks in without 50% improvement and no psychiatric review in 8 weeks. Dr. Farouk spends about 9 of her 20 weekly hours on caseload review. The Advancing Integrated Mental Health Solutions (AIMS) Center guide for four care managers is 8 to 12 hours, so protect a weekly review hour per care manager before adding psychiatry time.

   Owner **Program delivery lead and medical director** When **Next schedule cycle**

Interview mock-up prepared by Josiah Golles. Not an official CenterWell or Humana tool. All patients, providers, and figures are fictional and illustrative. Full dashboard on page 2.

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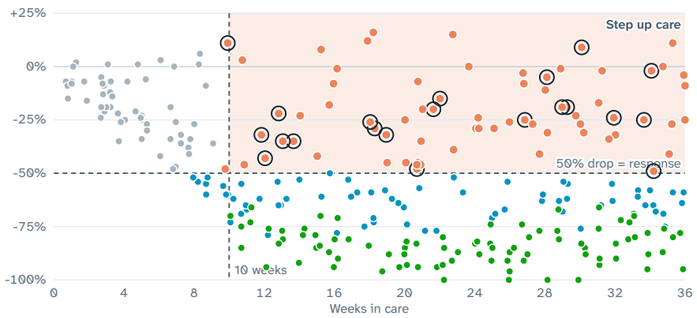
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Program reach (enrolled of eligible)	Referral to initial assessment	Caseload per care manager	Response at 6 months	Same-day suicide risk check	Cost PMPM vs. matched group
46 %	**16** days	**70** avg	**48** %	**97** %	**−7.6** %
Stretch target 50%	Target 14 days or less	Band 60 to 80, 1 above	Target 50% or more	Target 100% (31 of 32)	Target 5% lower or better
Watch	Off target	Watch	Watch	Act now	On target
12 months to Sep 30, 2026	Referrals Jul to Sep 2026	Active as of Oct 19, 2026	Enrolled Jan to Mar 2026	Jul to Sep 2026	12 months to Sep 2026. Range −1% to −14%

Treat-to-target: every active patient

Active patients as of Oct 19, 2026. Change in PHQ-9 from baseline by weeks in care. Patients in the shaded zone are 10+ weeks in and haven't improved 50%. They need a treatment change and a psychiatric case review.

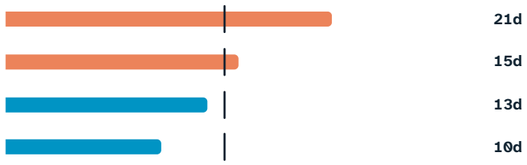
In remission (PHQ-9 under 5) **86** · Responding (50%+ drop) **67** · Under 10 weeks, not yet on target **56** · 10+ weeks, not on target **70** · No psychiatric review in 8 weeks



70 patients not on target at 10+ weeks **21** with no psychiatric review in 8 weeks

Days from referral to initial assessment

Median by care manager, referrals Jul 1 to Sep 30, 2026. Target is 14 days or less (black tick).



6 days Median to first contact, program

29 Reached, waiting for assessment now

17% Referred but unreached (target under 15%)

Screening to follow-up

Each bar is patients who reached that step. The gaps between steps are where patients drop out of care.

Depression screening and follow-up (HEDIS DSF-E)

12 months to Sep 30, 2026. A Star Ratings measure from measurement year 2027.

Seniors screened Rate 1, 3,164 of 4,020		**79%** Watch
Positives followed up in 30 days Rate 2, any follow-up care		**83%** On target

Tandem Care referral funnel Jul 1 to Sep 30, 2026

Positive (PHQ-9 or GAD-7 of 10+)		**214**
↓ 46 positive screens not referred		
Referred to Tandem Care		**168** 79% of prior
↓ 29 unreached after 3 attempts		
Reached by care manager		**139** 83% of prior
↓ 18 reached, declined or still waiting		
Enrolled (initial assessment)		**121** 87% of prior
↓ 17 no follow-up yet		
First follow-up within 30 days		**104** 86% of prior
↓ 16 follow-up without a symptom score		
Follow-up with PHQ-9 or GAD-7 scored		**88** 85% of prior

Scheduling and attendance

Initial assessments, Jul 1 to Sep 30, 2026.

138 Scheduled

121 Completed

12% No-show rate, target under 10%

How visits happened



No-show rate by visit type Target under 10% (black tick)



Clinical outcomes, safety, and experience

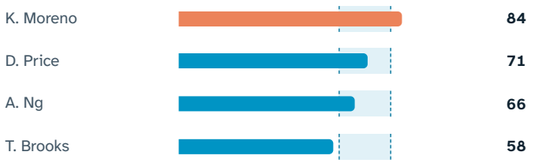
Bar is the current rate. Black tick is the target. Select a measure to see open gaps. Each measure lists its time period.

Response at 6 months 50%+ drop in PHQ-9. Enrolled Jan to Mar 2026		**48%** Watch
Remission at 6 months PHQ-9 under 5. Enrolled Jan to Mar 2026		**27%** Watch
Anxiety improvement at 6 months GAD-7 down 5+ points. Enrolled Jan to Mar 2026		**51%** On target
Same-day suicide risk check Positive PHQ-9 item 9 with a C-SSRS that day. Jul to Sep 2026		**97%** Act now
7-day follow-up after psychiatric stay HEDIS FUH. Year to date 2026		**52%** Off target
Patients who would recommend Tandem Care Patient survey, n=88. Jul to Sep 2026		**91%** On target
Antidepressant medication management HEDIS AMM, secondary. Year to date 2026		**67%** Watch

AMM is shown as a secondary measure because the National Committee for Quality Assurance (NCQA) is retiring it from health plan ratings.

Active caseload by care manager

Active patients as of Oct 19, 2026. Shaded band is the target range of 60 to 80 per full-time care manager.



4.0 Care manager FTE (LCSW)

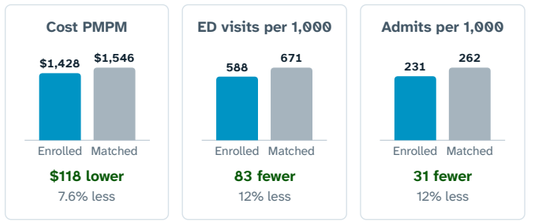
9 of 20 hrs Psychiatric consultant hours on caseload review, per week

8 to 12 hrs Advancing Integrated Mental Health Solutions (AIMS) Center guide for 4 care managers

Cost and utilization

12 months ending Sep 30, 2026. Patients enrolled 6+ months vs. a matched group who screened positive and didn't enroll.

Enrolled (n=184) · Matched, not enrolled (n=368)



Cost difference **7.6% lower**, likely range 1.2% to 13.8% lower

Updated quarterly. Claims lag about 30 days.

Medicare Star Ratings link

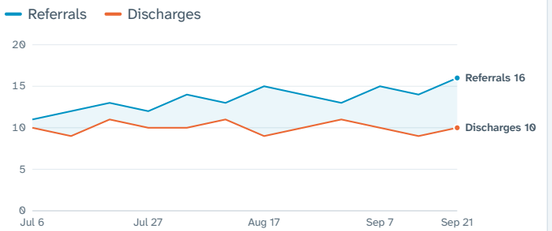
DSF-E becomes a Star measure in measurement year 2027. Improving or Maintaining Mental Health is triple-weighted in the 2027 Star Ratings.

Targets are placeholders until set with the Humana Stars team.

Depression screening and follow-up HEDIS DSF-E. 12 months to Sep 2026		**81%** On target
Improving or Maintaining Mental Health Health Outcomes Survey, plan level. 2026 results		**79%** Watch

New referrals vs. discharges per week

Weekly, Jul 6 to Sep 21, 2026. When referrals run above discharges, the caseload grows and waits get longer.



Net growth of 42 patients over 12 weeks, about 4 a week. At this pace the program adds a full care manager's caseload (70 patients) about every 5 months.

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